



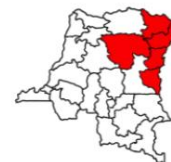
EBOLA PRESIDENTIAL FORCE TASK 17

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°118/MVEBDB/10/09/2026

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HIGHLIGHTS (last 24 hours)

At the end of the day on September 9, 2026, 99 new confirmed cases, including 29 community deaths, of the Disease Ebola virus (EV) cases have been recorded in the provinces of Ituri (47 cases), North Kivu (46 cases) and Haut-Kivu, respectively. Uélé (6). However, 10 intra-CTE deaths were also recorded. No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CASE 6,942	ACCUMULATIVE DEATH CONFIRMED 3,349	LETHALITY 48.2%	PATIENTS IN ISOLATION/CTE 823	ACCUMULATIVE HEALED 1,647	TRACKING RATES CONTACTS (Of the Day) 84.4%
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Affected Provinces and Affected Health Zones



6 Provinces Affected

Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo & Bas Uélé



61 out of 151 health zones affected (40.4%)



Summary of key points from the last 24 hours

For the day of September 9, 2026, **99 new confirmed cases** of Ebola Virus Disease (EVD) were reported, including **29 community deaths**.

The confirmed cases are distributed across the provinces of **Ituri (47 cases)**, **North Kivu (46)** and **Haut Uélé (6)**. In total, **10 intra-CTE deaths** were unfortunately recorded, including 7 in Ituri, 2 in North Kivu and 1 in Haut Uélé, bringing the **daily cumulative number of deaths to 39**.

On the other hand, **36 patients** were declared cured after obtaining two negative results for Ebola virus disease (32 in Ituri, 2 in Tshopo, 1 in Haut-Uele and 1 in North Kivu).

A total of **6,942 confirmed cases**, including **3,349 deaths**, have been recorded in the DRC since the start of the epidemic up to September 9, 2026, representing a case fatality rate of **48.2 %**. The epicenter remains the Ituri province, which accounts for **79.3%** of the confirmed cases reported across the six affected provinces, as well as **47.5%** of new cases confirmed in the last 24 hours.

No new health zones have been affected in the last 24 hours.

hours. Thus, **61 out of 151 health zones (40.4%)** remain affected in the 6 affected provinces. Ituri has 28 out of 36 affected health zones, followed by North Kivu (16/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34) and Bas-Uélé (3/11).

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 9, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	47	5508	2,501	45.4%	28/36 (77.8%)
North Kivu	46	1139	725	63.7%	16/34 (47.0%)
Haut-Uélé	6	264	110	41.7%	6/13 (46.1%)
Tshopo	0	24	9	37.5%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas Uélé	0	4	3	75.0%	3/11 (27.3%)
Total	99	6,942	3,349	48.2%	61/151 (40.4%)

Table 2. Distribution of confirmed cases and deaths by province and health zone, as of September 9, 2026.

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	5508	2501	45.4%	47	13	7	20
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	177	39	22.0%	1	0	0	0
Boga	2	2	100.0%				0
Bunia	1528	477	31.2%	5	1	0	1
Damascus	33	9	27.3%	1	0	0	0
Drodro	15	7	46.7%				0
Fataki	78	30	38.5%				0
Gethy	7	3	42.9%				0
Kambala	2	0	0.0%				0
Kilo	33	12	36.4%				0
Komanda	123	82	66.7%	5	2	0	2
Lita	251	123	49.0%	4	1	0	1
Logo	13	5	38.5%				0
Lolwa	31	8	25.8%	6	1	0	1
Mahagi	4	1	25.0%				0
Mambasa	17	7	41.2%				0
Mandima	44	24	54.5%				0
Mangala	309	146	47.2%	12	5	0	5
Mongbwalu	644	294	45.7%	4	1	0	1
Nia-Nia	229	116	50.7%	1	1	0	1
Nizi	726	273	37.6%	3	0	0	0
Nyankunde	125	36	28.8%				0
Rimba	11	4	36.4%				0
Rwampara	1004	349	34.8%	5	1		1

Tchomia	63	25	39.7%				0
To ventilate	N / A	415	N / A			7	7
North Kivu	1139	725	63.7%	46	14	2	16
Blessed	195	145	74.4%	10	7	0	7
Good	10	4	40.0%	1	0	0	0
Butembo	220	149	67.7%	3	1	0	1
Goma	1	0	0.0%				0
Kalunguta	23	11	47.8%	3	2	0	2
Katwa	486	309	63.6%	9	2	2	4
Kayna	1	0	0.0%				0
Kyondo	26	17	65.4%	1	0	0	0
Lubero	10	4	40.0%	3	0	0	0
Mabalako	18	8	44.4%	8	1	0	1
Manguredjipa	4	3	75.0%				0
Masereka	15	6	40.0%				0
Musienene	103	55	53.4%	2	0	0	0
Mutwanga	3	2	66.7%	1	0	0	0
Oicha	10	7	70.0%	1	1	0	1
Vuhovi	14	5	35.7%	4	0	0	0
Haut-Uélé	264	110	41.7%	6	2	1	3
Boma Mangbetu	29	14	48.3%				0
Gombari	3	1	33.3%				0
Isiro	92	38	41.3%	0	0	1	1
Pawa	52	25	48.1%	6	2		2
Rungu	2	1	50.0%				0
Wamba	86	31	36.0%				0
Tshopo	24	9	37.5%	0	0	0	0
Bafwasende	4	0	0.0%				0
Kabondo	4	1	25.0%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	10	5	50.0%				0
Mangobo	3	2	66.7%				0
Tshopo	1	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	4	3	75.0%	0	0	0	0
Buta	1	1	100.0%				0
Viadana	1	0	0.0%				0
Ganga	2	2	100.0%				0
Total	6942	3349	48.2%	99	29	10	39

*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

NA: Not Applicable.

** These deaths were recorded in the various CTEs of the Ituri province and are currently being assessed.

Table 3. Status of alerts notified by province, as of September 9, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	23	3	26	13	3	10	0	16	12
Bas Uélé	35	1	36	14	1	21	0	15	0
Ituri	824	77	901	146	73	561	1	219	115
North Kivu	826	39	865	94	32	586	0	126	71
South Kivu	7	0	7	1	0	4	0	1	1
Tshopo	103	3	106	6	3	97	0	9	6
Total General	1818	123	1941	274	112	1,279	1	386	205

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

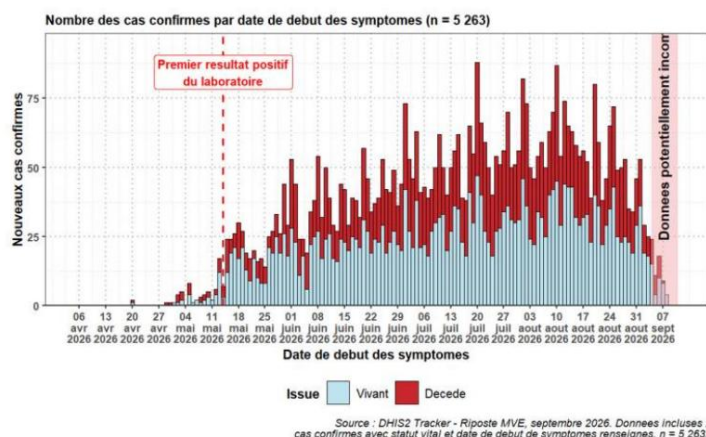
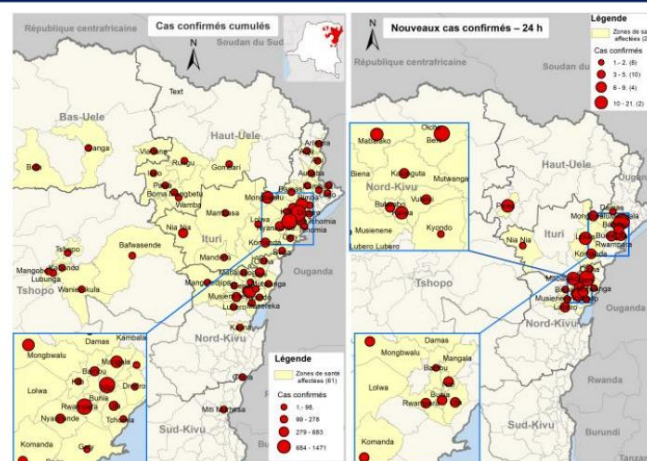


Figure 2. Spatial distribution of confirmed cases (24h and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

- Monitoring and support of response actions in all affected provinces.

2. RESPONSE ACTIONS BY PILLAR

1.1.1. Epidemiological surveillance

1.1.1.2. Main Actions

As of September 9, 2026, 1,941 alerts were reported in 6 provinces, 1,666 (85.8%) could be verified. All 386 alerts validated as suspected cases were investigated (100%) and 205 of them were transferred to CT/CTE (Table 3).

For the day of September 9, 2026, 19,945 out of the 23,630 under follow-up were seen, expressing a daily proportion of **84.4%** (below the threshold of 85%), a decline compared to the previous day (86.5%); 86.1% in North Kivu (9,131/10,600), 83.6% in Ituri (8,971/10,732), 80.3% in Bas Uélé (122/152), 83.5% in Tshopo (237/284) and 79.7% in Haut Uélé (1,484/1,862).

1.1.1.3. Challenges

Contact tracing is suboptimal in 4 out of 5 provinces with contacts currently being tracked.

1.2. Update on checkpoint and point of entry (PoC/PoE) activities

1.2.1. Main actions

As of September 9, 2026, four alerts were notified to the various PoE/PoCs, three **in North Kivu** and one in **Ituri**. In **North Kivu**, all three alerts were validated after investigation: a live alert from Mambasa to the Mukulia PoC (Beni Health Zone), referred to the CT, and two bodies intercepted at the Mukulia and Mavivi PoCs (Beni Health Zone), sent to the morgue for swab. In **Ituri**, a live alert from Basali to Bunia was validated at the Irumu PoC (Nyankunde Health Zone) and transferred to the Nyankunde CTE.

Table 4. Status of key indicators for PoC/PoE by province, as of September 9, 2026

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Haut-Uélé	Overall
Report completeness	91.7% (55/60)	100% (19/19)	55.5% (5/9)	94.4% (17/18)	64.0% (7/11)	88.0% (103/117)
% of functional strategic PoE/PoC	100%	100%	100%	100%	100%	100%
Number of people who switched to PoE/PoC % of	142 526 88 843		7,361	18,072	13,336	270,138
travelers screened	99.00% 98.17% 98.60%	99.00% 98.00%				98.20%
% of travelers who washed their hands	98.50% 98.17% 98.60%	98.80% 98.00%				98.00%
% of travelers made aware	99.10% 98.17% 100.00%	100.00% 99.00%				99.20%
Number of alerts notified	1	3	0	0	0	4
Number of validated live alerts (suspected cases)	1	1	0	0	0	2
Number of bodies intercepted today; % of suspected	0	2	0	0	0	2
cases transferred to CT/ CTE	100%	100%	N / A	N / A	N / A	100%
% of lifeless bodies swabed/ secured	N / A	100%	N / A	N / A	N / A	100%
% of suspected cases investigated within 24 hours	100%	100%	N / A	N / A	N / A	100%

1.2.2. Challenges

Operationalization of PoE/PoC remains suboptimal: low reporting completeness in South Kivu (55.5%, 5/9) and Haut-Uélé (64.0%, 7/11: 4 PoE/PoC without internet connection); persistent insecurity hindering the commissioning of the Avakubi PoC (Ituri) and continuation of the strike for non-payment of service providers of four PoCs (Nzebi, Galay, Base Yedi and Marabo); in Tshopo, 12 planned PoCs remain activated; no PoE/PoC report from Bas-Uélé.

1.3. Laboratory

1.3.1. Main actions

- **Ituri : 47 new positive results** (34 living and 13 deaths) out of 312 new samples received and analyzed (**positivity: 15.1%**).
- **North Kivu: 46 new positive results** (32 living and 14 dead) out of 139 samples analyzed (**positivity of 33.1%**).

- **Haut-Uélé: 6 new positive results** (4 living and 2 dead) out of 21 samples analyzed (**positivity: 28.6%**).
- **Bas-Uélé: 11 samples** received and tested (11 live), **all came back negative**.
- **Tshopo: 5 samples received and tested** (5 live), **all came back negative**.
- **South Kivu: 8 samples** received and tested (8 live), **all came back negative**

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri**, 24 of the 77 planned infection control zones (31.2%) were opened, 184 of the 184 identified households were decontaminated (100%), and all 5 health facilities (100%) were decontaminated; no infection prevention and control (IPC) kits were distributed to households. Regarding the health and safety surveys (HSS), 96 alerts were recorded (including 7 carried over from the previous day), 77 HSS were carried out (80.2%), and 65 bodies were swabed.
- **In North Kivu**, 47 rings were opened out of the 54 expected (87.0%), 1 household PCI kit was distributed in Manguredjipa, and 15 FOSA assessments were carried out with an average score of 42.5%, among other things. in the health zones of Beni, Masereka, Manguredjipa and Kalunguta; risk assessment of staff exposure: 42/67 at high risk (Beni, Kyondo, Biena and Masereka); closure of the PCI/WASH training of the 30 management teams of Butembo and Katwa and dispatch of a batch of PCI/WASH inputs to the health zone of Biena; the EDS pillar received 40 alerts (21 reports from the previous day), carried out 40 EDS with swabs (100%).
- **In Tshopo**, decontamination of 5 rooms, 1 latrine and 1 shower at the household of the 22nd case Confirmed (Neema Health Center, Makiso-Kisangani Health Zone) with incineration of belongings; provision of infection prevention and control (IPC) supplies to Tshopo General Referral Hospital, Mangobo General Referral Hospital, and Williams Clinic; monitoring and support of 12 community health workers in 5 health zones with briefings for 128 service providers; 8 triage units installed out of 12 expected and 500 liters of water supplied to Wanie-Rukula. The blood and health data (BHD) pillar received 3 alerts (2 of which were postponed), no BHDs were carried out, and 2 bodies could not be swabed.
- **In Haut-Uélé**, joint actions CREC-PCI for the implementation of the EDS in the AS of Neisu (ZS of Isiro), the support of the family of the deceased having been obtained after negotiation.

1.4.2. Challenges

- **In Ituri** : low ring road opening rate (31.2%; 24/77); insufficient decontamination teams in Mangala and Komanda; lack of mobility in Bambu, Mandima and Mangala; insufficient PCI inputs in Mandima.
- **In North Kivu** : Need to activate, train and motivate decontamination teams in uncovered health zones (Biena, Masereka, Kyondo, Mabalako and Vuhovi); lack of traceability of some households eligible for decontamination; lack of mobility for deployment of provincial PCI teams in the field; urgent need for training for all PCI Data on data collection and management tools.
- **In Tshopo** : insufficient PCI inputs in ESS, untrained service providers and the incinerator at the Makiso-Kisangani General Referral Hospital remains out of order.

1.5. Holistic care

1.5.1. Main actions

- **In Ituri**, there were 92 new admissions to the various dedicated facilities, compared to 106 discharges, including 32 recoveries. Thus, 457 patients are hospitalized in 989 beds, representing an occupancy rate of 46.2%; 407 out of 434 (94%) patients received hot meals; 14 out of 14 (100%) were children.

are under LNPE/SLM at the CTE, 3/3 (100%) at the crèche and 27/27 (100%) in the community.

- **In North Kivu**, 72 new admissions and 68 discharges were recorded, including 1 recovery; 291 Patients are hospitalized, representing an overall occupancy rate of overcapacity (132.2%; 291/220 beds available); 79/79 accompanying persons received hot meals (100%), as did 179/179 frontline providers. • **In Haut-Uélé**, there were 3 new admissions and 16 discharges (1 recovered). Thus, 54 patients are hospitalized out of 134 beds, representing an occupancy rate of 40.3%; in addition, 54 patients, 64 accompanying persons, and 26 primary healthcare providers received three hot meals; the INRB team arrived for prophylactic and specific treatment of Ebola virus disease.
- **In Tshopo**, no new admissions were recorded against 4 discharges (2 recovered). Three (3) patients are in isolation out of 25 beds, representing an occupancy rate of 12.0%.
- **In South Kivu**, no new admissions, 18 suspected cases remain in isolation.

1.5.2. Challenges

Treatment still partially substandard and limited capacity: **In Ituri**, the Ebola Treatment Centers (ETCs) are saturated with suspected cases: Nizi, Mambasa Lolwa, Kilo, Mangala, and Mandima; 30 patients are in critical condition. **In Haut-Uélé**, the Pawa ETC lacks hygiene facilities, and the Isiro ETC lacks space, water, and a constant supply of electricity .

In Tshopo, the lack of safe blood and limited technical resources are hindering patient care. **In Bas-Uélé**, the absence of TSE/CT centers and ambulances limits the isolation of cases.

1.6. Vaccination

1.6.1. Main actions

- **In Tshopo**, 1,779 people have been vaccinated to date out of 11,000 targeted TPL/PPL (16,2 %), including 1,142 in Makiso-Kisangani, 291 in Kabondo, 271 in Mangobo and 75 in Tshopo; 2,500 doses were deployed in the health zones, with the balance settling at 721 doses at the zone level and 2,000 frozen doses at the Hub depot.
- **In Ituri**, preparations for the vaccination of PPL continue in Bunia

1.6.2. Challenges

Insufficient doses are available **in Tshopo** (4,500 received out of 11,000 requested), there is a lack of mobility for contact tracing (3 vehicles required for the mobile strategy), and the Bafwasende vaccination site remains inactive. **In Bas-Uélé**, the stockout of ERVEBO in Buta has led to a temporary suspension of vaccinations for PPL and TPL.

1.7. Continuity of Care

- **In Tshopo**, unloading of the 3rd PPL at the CTE and 7 PPL under treatment (4 in Kabondo and 3 in Mangobo); monitoring of the delivery of essential generic medicines to the Lubunga Health Zone and monitoring of PPL; 116 cases received experimental treatment, including 13 confirmed and 103 under post-exposure prophylaxis (60 treatments completed, 11 interrupted and 32 ongoing); 2 recovered patients joined the response teams.

1.8. Risk communication and community engagement

1.8.1. Main actions

- **In Ituri**, 28,415 home visits reached 43,609 people and several educational talks reached 67,639 people; 150 mass awareness actions reached 10,626 people; approximately 1,200 people were sensitized at the market in the Kunda group and 1,600 students and 50 teachers in 3 schools in the Rwampara health zone; 258 community alerts were notified.

- **In North Kivu**, 44,882 people were reached during 14,695 home visits, 2,528 people were reached during 110 educational talks, 26 mass communication actions reached 5,593 people, 2 community dialogues and 5 advocacy efforts reached 45 and 5 people respectively; 210 community alerts were notified out of 450 expected (46.6%), 4 feedback issues resolved out of 7 (57.1%) and 2 incidents resolved out of 5 (40%); 79 members of pressure groups were mobilized in Manguredjipa and 36 political-administrative authorities and civil society actors met in the Nduko AS (Lubero Health Zone).
- **Haut Uélé** : 3 mass activities reached 569 people and 1 educational talk reached 33 people; intensified awareness-raising on the Wamba-PK18 axis by His Excellency the Vice-Governor accompanied by the response team; working session between the SGI coordination and the Red Cross; 11 community alerts notified and 2 feedbacks resolved.
- **In Tshopo**, 8 mass awareness campaigns reached 2,938 people in the 5 health zones of the city of Kisangani and 2 educational talks reached 35 people in Bafwasende and Mangobo; 1,432 traders were sensitized in the 3 municipal markets of Kabondo and 1,506 people in 3 schools in Makiso; 2 interactive programs were hosted and 12 spots broadcast in 6 press outlets; 36 community alerts were notified as well as 2 feedbacks, all resolved.

1.8.2. Challenges

Persistent community resistance: **In Tshopo**, an attack against teams was recorded in the Bethsaïda health district (Mangobo health zone) amid rumors of an "Ebola business"; 11 contacts are refusing follow-up in the Matete health district (Mangobo health zone); and a refusal to provide samples was resolved through negotiation in Kumbakisaka. **In Haut-Uélé**, a refusal to participate in the census persists in Tely (Isiro health zone); rumors are circulating that patients are being held in tents at the Isiro General Referral Hospital, and posters and leaflets have been destroyed. **In Ituri**, resistance to the Ebola Health Distribution Teams (EDTs) has been reported in Bunia, Nizi, Nia-Nia, and Aru. **In North Kivu**, a confirmed case escaped after confirmation in Masereka, and a family in Munzambaye (Butembo health zone) is refusing decontamination of their household.

1.9. Mental Health and Psychosocial Support (MHPSS)

1.9.1. Main actions

- **In Ituri**, 17 of the new confirmed cases and 113 of the 164 suspected cases received SMSPS interventions; 60 results were announced, including 17 positive cases, 46 PPL supported and 100 households assisted, 52 children and 12 recoveries monitored in the community.
- **In North Kivu**, 27 people among the new confirmed cases and 145 confirmed cases under follow-up were taken care of, as well as 115 new suspects; 101 results announced, including 27 positive, 34 PPL supported, 46 households accompanied, 14 of the 14 cured, i.e. 100% followed up; 399 people (146 men, 186 women and 67 children) benefited from psychoeducation sessions.
- **In Tshopo**, 2 confirmed cases under follow-up, 1 new suspected case and 2 suspected cases under follow-up benefited from SMSPS interventions; 2 announcements of negative results were made; 6 contacts were supported at the CT of the Makiso-Kisangani General Referral Hospital, 2 suspected cases were prepared for sampling and 94 people benefited from psychoeducation, including 93 members of the Kabondo Health Zone community on the importance of vaccination.
- **In Haut-Uélé**, 2 new confirmed cases, 39 confirmed cases under follow-up, 5 new suspected cases and 12 suspected cases under follow-up benefited from SMSPS interventions, as well as 93 accompanying persons at the CTE and 66 PPL; 9 announcements of results including 2 positive; training of 20 APS in Boma-Mangbetu and 5 recovered from the CTE in Wamba are awaiting unloading.

1.9.2. Challenges

• In Ituri, low coverage of SMSPS interventions around the latest confirmed cases in Ituri (Komanda and Lita). In North Kivu, low coverage of SMSPS interventions around the latest confirmed cases in the health zones of Biena, Manguredjipa, Kayna, Lubero, Masereka, Kalunguta, Mabalako, Mutwanga, and Vuhovi; absence of nurseries in 10 affected health zones; insufficient human resources and motivation in 10 health zones. In Haut-Uélé, insufficient staff and means of transport (Boma Mangbetu and Wamba) and absence of SMSPS providers in Tshopo (Bafwasende and Wanie-Rukula).

1.10. Logistics

1.10.1. Main actions

- In Haut-Uélé, delivery of a table and 4 chairs to the surveillance pillar for the installation of an office at the CTE of Isiro and monitoring of the installation work of this CTE; deployment of a vehicle in the ZS of Pawa for a mission of the database pillar; fuel management: previous balance of 1,160 liters, withdrawals of 440 liters, i.e. a current balance of 720 liters.
- In North Kivu, 23 calls were received, resulting in 23 successful dispatches (100%), and 8 ambulances were mobilized out of 11 available (73%); 108 patients (57 suspected and 51 confirmed), 79 caregivers, and 179 frontline workers received a hot meal (100%); 1 orphaned infant is receiving breast milk substitute, and no separated children are being cared for at the orphanage.
- In Tshopo, a freezer and refrigerator were received for the laboratory, followed by the construction of the Bafwasende CTE (Center for Treatment and Evaluation), support for the deployment of IPC (Infection Prevention and Control) kits to the health zones of Tshopo and Mangobo, and monitoring of the distribution of PHATKIN medication. In Bas-Uélé, no data was transmitted in the last 24 hours.

1.10.2. Security

1.10.3. Main actions

- In North Kivu, exchange between the risk mitigation pillar and the head of the civil protection branch on the passage of lifeless bodies during the night at the different points of entry; support and security coverage, all carried out at 100%, of the EDS teams for the burial of confirmed cases in the cemeteries of Bundji, Kyanzaba, Kimbangu and Masiani, and of the PCI teams for the decontamination of households and care structures in the AS of Tamende and Mangote (CM Furaha, CM Emmanuel, CM Banalisasa, Lukogho and Solferino dispensaries).

1.10.4. Challenges

In North Kivu, there is a lack of a permanent vehicle for the security team and a lack of financial incentive for the agents.

1.11. PSEA (Prevention of Sexual Exploitation and Abuse)

1.11.1. Main actions. • In Ituri,

ensure awareness-raising on sexual exploitation and abuse in the response teams; 124 response staff members (70 men and 54 women) received the PSEA briefing; 268 people (122 men and 146 women) were sensitized in the community.

1.11.2. Challenges

- Still insufficient coverage of PSEA activities, with only 13 of the 36 health zones in Ituri (36.1%) aware of the reporting mechanism; lack of mapping of stakeholders and refusal of some partners to align; insufficient mass awareness materials; lack of computer kits and office supplies for the pillar.

KEY MESSAGES

- Performance remains strong in epidemiological investigations (**100% of suspected cases investigated**) while the daily proportion of contact tracing (**84.4%**) is declining compared to the monitoring (86.5%) and is below the threshold of 85.0%.
- Sustained transmission in the provinces of Ituri, North Kivu, and Haut-Uélé, with Tshopo province showing sporadic reports. However, North Kivu province presents a worrying profile well beyond its characteristic high lethality rate, with a growing proportion (**46.5%**) of new confirmed cases approaching that of Ituri.
- In total, **99 new confirmed cases**, including **29 community deaths and 10 intra-CTE deaths**, were recorded in the last 24 hours, bringing the national total to **6,942 confirmed cases and 3,349 deaths (case fatality rate: 48.2%)**.
- Ituri remains the epicenter of the epidemic, concentrating **79.3%** of cumulative confirmed cases and **47.5%** of new cases reported in the last 24 hours, with North Kivu getting closer (**46.5%**).
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains: Rapidly increase reception capacities in CT/CTE in North Kivu.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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